

§ 170.315(g)(9)

Application access — all data request

Certification Companion Guide v1.5

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This Certification Companion Guide (CCG) is an informative document designed to assist with health IT product certification. The CCG is not a substitute for the requirements outlined in regulation and related ONC final rules. It extracts key portions of ONC final rules' preambles and includes subsequent clarifying interpretations. To access the full context of regulatory intent please consult the [Certification Regulations](#) page for links to all ONC final rules or consult other regulatory references as noted. The CCG is for public use and should not be sold or redistributed.

Archived Version: [§ 170.315\(g\)\(9\) Application access — all data request CCG](#)

Certification Clarifications

Technical Explanations and Clarifications

- *Security:*
 - For the purposes of certification there is no conformance requirement related to the registration of third-party applications that use the application programming interfaces (APIs). If a Health IT module requires registration as a pre-condition for accessing the APIs, then, the process must be clearly specified in the API documentation as required by the criterion. ONC strongly believes that registration should not be used to block information sharing via APIs.
 - This criterion does not currently include any security requirements beyond the privacy and security approach detailed above, but ONC encourages organizations to follow security best practices and implement security controls, such as penetration testing, encryption, audits, and monitoring as appropriate. ONC expects health IT developers to include information on how to securely use their APIs in the public documentation required by the certification criteria and follow industry best practices. [see also [80 FR 62676](#) and [85 FR 25642](#)]

- [ONC strongly encourages developers to build security into their APIs following industry best practices. \[see also 80 FR 62677\]](#)
 - A “trusted connection” means the link is encrypted/integrity protected according to § 170.210(a)(2) or (c)(2). As such, ONC does not believe it is necessary to specifically name HTTPS and/or SSL/TLS as this standard already covers encryption and integrity protection for data in motion. [see also [80 FR 62676](#)]
 - APIs could be used when consent or authorization by an individual is required. In circumstances where there is a requirement to document a patient’s request or particular preferences, APIs can enable compliance with documentation requirements. The Health Insurance Portability and Accountability Act of 1996 Privacy Rule (45 CFR Part 160 and Part 164, Subparts A and E) permits the use of electronic documents to qualify as writings for the purpose of proving signature, e.g., electronic signatures. [see also [80 FR 62677](#)]
- The audit record should be able to distinguish the specific user who accessed the data using a third-party application through the certified API to meet the requirements of § 170.315(d)(2) or (d)(10).
- A health IT developer must demonstrate the API functionality of the Health IT Module properly performs consistent with this certification criterion’s requirements. As part of the demonstration process, a health IT developer is permitted, but is not limited, to using existing tools for creating its own app or “client” to interact with the API or using a third-party application for demonstration.
- By requiring that documentation and terms of use be open and transparent to the public by requiring a hyperlink to such documentation to be published with the product on the ONC Certified Health IT Product List, ONC hopes to encourage an open ecosystem of diverse and innovative applications that can successfully and easily interact with different Health IT Modules’ APIs. [see also [80 FR 62679](#) and [85 FR 25642](#)]
- Consistent with Executive Order (EO) 14168 and OPM guidance, Health IT Modules certifying and/or currently certified to certification criteria that cross-reference the USCDI standard at 45 CFR 170.213 are only required to demonstrate the capability to categorize data on individuals for the sex data element in accordance with the following SNOMED CT® codes:
 - 248152002 |Female (finding)| and
 - 248153007 |Male (finding)|
- Further, these Health IT Modules are no longer required to support the following USCDI data elements for purposes of certification:
 - Sex parameter for clinical use in USCDI version 5;
 - Name to use in USCDI version 5;
 - Pronouns in USCDI version 5.
- Through the [C-CDA Patch Process](#), the HL7® Structured Documents Work Group (SDWG) approves C-CDA “patches”, which are corrections for issues with the C-CDA implementation guide (C-CDA IG) and companion guides. C-CDA “patches” include corrections for issues such as ambiguous requirements and requirements incompatible with real world deployment. Similar to C-CDA “patches” are C-CDA “additional guidance”. C-CDA “additional guidance” approved by the SDWG indicates guidance included in a newer version of the C-CDA IG or companion guide as being relevant to a previous version of the C-CDA IG or companion guide. A C-CDA “patch” may require a code change to correct errors or ambiguities in the guide, while “additional guidance” is purely clarifying and does not require any code changes. Though C-CDA “patches” and “additional guidance” are not required for

certification purposes (unless indicated in the Certification Companion Guide), health IT developers may optionally implement C-CDA “additional guidance” in their Health IT Module and still be conformant with § 170.315(g)(9) criterion requirements.

- More information regarding the C-CDA Patch Process and C-CDA “patches” and “additional guidance” approved by the SDWG is available on the HL7® Confluence pages of [C-CDA 'Patch' Process](#) and [Approved Patches and Additional Guidance](#).

Technical outcome – The API must be able to respond to requests for patient data (using an ID or other token) for all of the data categories specified in the United States Core Data for Interoperability Standard (USCDI) at one time in a summary record formatted according to the C-CDA Release 2.1 Continuity of Care Document (CCD) template.

- Please refer to the USCDI for the data standards that are required.
- The technology specifications should be designed and implemented in such a way as to return meaningful responses to queries, particularly with regard to exceptions and exception handling, and should make it easy for applications to discover what data exists for the patient. [see also [80 FR 62678](#)]
- The term “token” that is used here is not to be interpreted as the token in the OAuth2 workflow, but simply as something that would uniquely identify a patient.
- In order to mitigate potential interoperability errors and inconsistent implementation of the HL7 CDA® R2 Implementation Guide: Consolidated CDA Templates for Clinical Notes, Release 2.1, ONC assesses, approves, and incorporates corrections as part of required testing and certification to this criterion. [see also [Health IT Certification Program Overview](#)] Certified health IT adoption and compliance with the following corrections are necessary because they implement updates to vocabularies, update rules for cardinality and conformance statements, and promote proper exchange of C-CDA documents. There will be a 90-day delay from the time the CCG has been updated with the ONC-approved corrections to when compliance with the corrections will be required to pass testing (i.e., C-CDA 2.1 Validator). There will be an 18-month delay before a finding of a correction’s absence in certified health IT during surveillance would constitute a non-conformity under the Certification Program.

Technical outcome – The API must be able to respond to requests for patient data associated with a specific date as well as with a specific date range.

- Health IT returning an entire patient record that does not reflect the specific date or date range requested is not permissible when a specific date or date range is requested. [see also [80 FR 62678](#)]
- The health IT developer can determine the method and the amount of data by which the health IT uniquely identifies a patient. [see also [80 FR 62678](#)]
- The API must be able to send, at minimum all required data for a specified date range(s). ONC acknowledges that there will be organizational policies and/or safety best practices that will dictate additional data to be sent and when data is considered complete and/or ready for being sent. This should be appropriately described in the API documentation.
- The approach used to provide the CCD document(s) is set by the API. An approach based on providing one or more CCD documents matching to the patient's selected date or date range is a valid approach.

Technical outcome – The API must include accompanying documentation which contains, at a minimum:

- API syntax, function names, required and optional parameters and their data types, return variables and their types/structures, exceptions and exception handling methods and their returns;
 - Software components and configurations that would be necessary for an application to implement in order to be able to successfully interact with the API and process its response(s); and
 - The terms of use for the API, including, at a minimum, any associated developer policies and required developer agreements.
- No additional clarifications.

Technical outcome – The documentation used to meet the provisions in (g)(9)(ii)(A)(1)-(3) must be available through a publicly accessible hyperlink.

- The hyperlink provided for all of the documentation referenced by provision (g)(9)(ii)(A) must reflect the most current version of the health IT developer's documentation.
- All the documentation referenced by provision (g)(9)(ii)(A) must be accessible to the public via a hyperlink without additional access requirements, including, without limitation, any form of registration, account creation, "click-through" agreements, or requirement to provide contact details or other information prior to accessing the documentation.

Regulation Text

§ 170.315 (g)(9) Application access – all data request—

The following technical outcome and conditions must be met through the demonstration of an application programming interface.

(i) Functional requirements.

(A) -

- (1) Respond to requests for patient data (based on an ID or other token) for all of the data classes expressed in the standards in § 170.213 at one time and return such data (according to the specified standards, where applicable) in a summary record formatted in accordance with § 170.205(a)(4) and (5) following the CCD document template, and as specified in paragraphs (g)(9)(i)(A)(3)(i) through (iv) of this section for the time period up to and including December 31, 2025, or
- (2) Respond to requests for patient data (based on an ID or other token) for all of the data classes expressed in the standards in § 170.213 at one time and return such data (according to the specified standards, where applicable) in a summary record formatted in accordance with § 170.205(a)(4) and (6) following the CCD document template, and as specified in paragraphs (g)(9)(i)(A)(3)(i) through (iv) of this section.
- (3) The following data classes:
 - (i) *Assessment and plan of treatment*. In accordance with the “Assessment and Plan Section (V2)” of the standards specified in § 170.205(a)(4); or in accordance with the “Assessment Section (V2)” and “Plan of Treatment Section (V2)” of the standards specified in § 170.205(a)(4).
 - (ii) *Goals*. In accordance with the “Goals Section” of the standard specified in § 170.205(a)(4).
 - (iii) *Health concerns*. In accordance with the “Health Concerns Section” of the standard specified in § 170.205(a)(4).

- (iv) *Unique device identifier(s) for a patient's implantable device(s).* In accordance with the “Product Instance” in the “Procedure Activity Procedure Section” of the standard specified in § 170.205(a)(4).
 - (B) Respond to requests for patient data associated with a specific date as well as requests for patient data within a specified date range.
- (ii) Documentation.
- (A) The API must include accompanying documentation that contains, at a minimum:
 - (1) API syntax, function names, required and optional parameters and their data types, return variables and their types/structures, exceptions and exception handling methods and their returns.
 - (2) The software components and configurations that would be necessary for an application to implement in order to be able to successfully interact with the API and process its response(s).
 - (B) The documentation used to meet paragraph (g)(9)(ii)(A) of this section must be available via a publicly accessible hyperlink.

Standards & References

The following standards are referenced within the certification criteria for:
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§ 170.213(b)

[United States Core Data for Interoperability, Version 3 \(USCDI v3\)](#) 

Referenced in the Following:

- Paragraph (g)(9)(i)(A)
-

SVAP-approved Versions:

§ 170.205(a)(4)

[HL7 Implementation Guide for CDA® Release 2: Consolidated CDA Templates for Clinical Notes \(US Realm\), Draft Standard for Trial Use, Volume 1—Introductory Material, Release 2.1 and HL7 Implementation Guide for CDA® Release 2: Consolidated CDA Templates for Clinical Notes \(US Realm\), Draft Standard for Trial Use, Volume 2—Templates and Supporting Material, Release 2.1](#) [↗](#)

Referenced in the Following:

- Paragraph (g)(9)(i)(A)
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SVAP-approved Versions:

[HL7 CDA® R2 Implementation Guide: Consolidated -CDA Templates for Clinical Notes Edition 4.0 - US Realm](#) [↗](#)

§ 170.205(a)(6)

[HL7® CDA® R2 Implementation Guide: C-CDA Templates for Clinical Notes STU Companion Guide, Release 4.1—US Realm](#) [↗](#)

Referenced in the Following:

- Paragraph (g)(9)(i)(A)
-

SVAP-approved Versions:

[HL7 CDA® R2 Implementation Guide: Consolidated -CDA Templates for Clinical Notes Edition 4.0 - US Realm](#) [↗](#)

Certification Dependencies

§ 170.405

Real World Testing

Products certified to this criterion must complete requirements outlined for the Real World Testing Conditions and Maintenance of Certification.

Note: Enforcement discretion has been issued for CY 2025–2026. Refer to the [Enforcement Discretion Notices](#) for full details.

Privacy and Security

This certification criterion was adopted at § 170.315(g)(9). As a result, an ONC Authorized Certification Body (ONC-ACB) must ensure that a product presented for certification to this criterion includes the privacy and security criteria (adopted in § 170.315(d)) within the overall scope of the certificate issued to the product.

- The privacy and security criteria (adopted in § 170.315(d)) do not need to be explicitly tested with this specific criterion unless it is the only criterion for which certification is requested.
- As a general rule, a product presented for certification only needs to be tested once to each applicable privacy and security criterion (adopted in § 170.315(d)) so long as the health IT developer attests that such privacy and security capabilities apply to the full scope of capabilities included in the requested certification. However, exceptions exist for § 170.315(e)(1) “View, download and transmit to 3rd party (VDT)” and (e)(2) “Secure messaging”, which are explicitly stated.

For more information on the approaches to meet these Privacy and Security requirements, please download the Privacy and Security CCG below:

Approaches:

if choosing

Approach 1

- [Authentication, access control, and authorization \(§ 170.315\(d\)\(1\)\)](#).
- [Trusted connection \(§ 170.315\(d\)\(9\)\)](#).
- Either [Auditable events and tamper-resistance \(§ 170.315\(d\)\(2\)\)](#) or [Auditing actions on health information \(§ 170.315\(d\)\(10\)\)](#).
 - Note: If a developer elects to demonstrate §170.315(d)(2), they only need to demonstrate § 170.315(d)(2)(i)(A) – (B) and § 170.315(d)(2)(ii) – (v) for the privacy and security requirements for this criterion.
- [Encrypt authentication credentials \(§ 170.315\(d\)\(12\)\)](#).
- [Multi-factor authentication \(MFA\) \(§ 170.315\(d\)\(13\)\)](#).

if choosing

Approach 2

For each applicable privacy and security certification criterion not certified for Approach 1, the health IT developer may certify using system documentation that is sufficiently detailed to enable integration such that the Health IT Module has implemented service interfaces for each applicable privacy and security certification criterion that enable the Health IT Module to access external services necessary to meet the requirements of the privacy and security certification criterion. Please see the *21st Century Cures Act: Interoperability, Information Blocking, and the ONC Health IT Certification Program Final Rule* at [85 FR 25710](#) for additional clarification.

Design and Performance

The following design and performance certification criteria (adopted in § 170.315(g)) must also be certified in order for the product to be certified.

- [Quality management system \(§ 170.315\(g\)\(4\)\)](#): When a single quality management system (QMS) is used, the QMS only needs to be identified once. Otherwise, the QMS' need to be identified for every capability to which it was applied.

- [Accessibility-centered design \(§ 170.315\(g\)\(5\)\)](#): When a single accessibility-centered design standard is used, the standard only needs to be identified once. Otherwise, the accessibility-centered design standards need to be identified for every capability to which they were applied; or, alternatively, the developer must state that no accessibility-centered design was used.
- [Consolidated CDA creation performance \(§ 170.315\(g\)\(6\)\)](#): Consolidated Clinical Document Architecture (C-CDA) creation performance (§ 170.315(g)(6)) does not need to be explicitly tested with this criterion unless it is the only criterion within the scope of the requested certification that includes C-CDA creation capabilities. Note that the application of § 170.315(g)(6) depends on the C-CDA templates explicitly required by the C-CDA-referenced criterion or criteria included within the scope of the certificate sought. Please refer to the C-CDA creation performance CCG for more details.

Certification Companion Guide Changelog

The following changelog applies to:

§ 170.315(g)(9)Application access — all data request

Revision History

Version

1.0

Description of Change

Initial publication

Version Date

03-11-2024

Version

1.1

Description of Change

Standards Referenced updated to reflect 2024 Approved SVAP Standards.

Version Date

08-19-2024

Version #

1.2

Description of Change

For entire criterion, added clarification regarding compliance with EO 14168 and OPM guidance

Version Date

03-27-2025

Version #

1.3

Description of Change

Added clarification for entire criterion providing information regarding C-CDA patch process. Added update to Certification Dependencies section to reflect recent Real World Testing enforcement discretion notice.

Version Date

07-23-2025

Version #

1.4

Description of Change

Standards Referenced updated to reflect 2025 Approved SVAP Standards

Version Date

08-29-2025

Version #

1.5

Description of Change

Removal of requirements that expired on January 1, 2026.

Version Date

05-05-2026

